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Social Policy and Intervention with Chemically Dependent Women and Their Children

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ABSTRACT: The increase in drug dependency among infants has significant implications for social service providers and must be addressed on the policy level. The policy debates and legislation which have surfaced to date need to be examined more closely and considered in the context of social service needs. This article considers policy options from the framework of social needs and individual needs in relation to the problem of maternal and infant chemical dependency.

The issues of drug use, drug addiction, and drug production are not new. The use of drugs for medicinal, religious, and recreational purposes has existed for thousands of years (O'Brien & Cohen, 1984). In recent years, the non-medical use of drugs has received a great deal of attention. Controversy exists around the extent to which drug use and abuse have risen, and what those trends mean for policymakers. Data do indicate that among women of childbearing age, the use and abuse of illicit drugs have increased and it is a problem of national importance.

As has been discussed in the previous articles, the incidence, extent, and consequences of maternal drug use have increased and worsened in recent years. According to the National Institute on Drug Abuse (1989), by the age of 25, over 80% of today's young adults have tried an illicit drug. Over 5 million women of childbearing age (15-44) currently use an illicit drug. The National Drug Control Strategy reported an estimated 100,000 infants are exposed to cocaine each year, while the National Association for Perinatal Addiction Re-

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search and Education estimated as many as 375,000 infants are exposed to drugs each year (U.S. General Accounting Office, 1990b).

Some research indicates that the incidence of illicit drug abuse during pregnancy may be as high as 11% (Chasnoff, 1989). While the incidence may be high, the likelihood of diagnosis is low. Lack of early medical care and screening coupled with underreporting suggest that the problem is more prevalent than current research indicates. Social factors such as changes in gender role, occupational status, social expectations as well as risk factors such as poverty and single parenthood seem to contribute to the increase in drug use among women (Segal, 1988). Risk factors such as poor prenatal nutrition, poor prenatal care, and chaotic home conditions have been found to be linked with maternal drug abuse and have an affect on maternal responsiveness after the infant is born (Johnson & Rosen, 1990). Thus, it becomes difficult to separate the environmental stresses from the personal stresses, and to determine whether drug abuse is a symptom or a cause.

The increase in drug dependency among infants has significant implications for social service providers and must be addressed on the policy level. The policy debates and legislation which have surfaced to date need to be examined more closely and considered in the context of social service needs. This article considers policy options from the framework of social needs and individual needs in relation to the problem of maternal and infant chemical dependency.

Policy Options

There tend to be three areas of focus when discussing policy alternatives to address the problem of drug abuse: 1) enforcement and punishment, 2) treatment, and 3) education for prevention. From a policy perspective, these three options require very different legislative approaches. The direction of social services and the implications for social work practice depend on the policy approach chosen.

The enforcement and punishment approach relies on the incarceration of those who produce, supply, sell, and/or consume illegal substances. The goal is to eradicate both those who are involved and the substances and thereby put an end to the problem. In 1987, the United Nations International Conference on Drug Abuse and Illicit Trafficking was held with 138 nations participating. Their goal was to better coordinate anti-drug activities world-wide (General Account-

ing Office, 1990a). The Conference revealed that unlike many other nations, United States efforts include use of the military for the interdiction of drugs in producing nations as well as coming into this country. This approach is reflected in the common terminology used, that we are "fighting a drug war." While enforcement and control of illegal drugs are important, this approach to fighting a war necessitates an enforcer and an enemy, and thus overshadows the other policy alternatives.

Treatment of drug users takes a different approach. When treating people who are addicted to drugs, a medical model is often reflected. There are no enforcers and enemies, but rather those who can help and those in need of service. The medical model for treatment reflects the belief that drug addiction is a disease, and can therefore be "cured" by medical professionals. By placing drug abuse as a disease to be diagnosed and treated, we "shift the emphasis from social and cultural to individual causes and cures" (Peele, 1989, p. 243). The result is that emphasis is placed on making individual change rather than examining and questioning the impact of social surroundings.

However, evidence suggests that treatment programs can help women to overcome their addiction to drugs (U.S. House Select Committee on Children, Youth, and Families, 1990b). Case management programs have been found to be particularly beneficial. Case managers can go into communities and help to link drug dependent women to services which may include pre or postnatal care, transportation, child care, or job training, in addition to substance abuse treatment services (Atkins, 1990).

The third approach stresses prevention. Through means such as education and public awareness campaigns, emphasis is placed on educating people about the negative consequences of drug use and possibly offering alternative ways of dealing with precipitators to drug use. Prevention can encourage closer examination of how social surroundings influence drug abuse, or it can rely on scare tactics such as those often seen in the media which portray the deadly side effects of drugs.

The direction of social policy and social services depends on which of these three approaches one holds. This is particularly true when discussing maternal use of illicit drugs where there is an alleged "willing" user, the mother, and an alleged "innocent and powerless" user, the infant. The approach which seems appropriate for dealing with the abusing mother may not be advantageous to the child, and vice versa. For example, punishing the mother by removing her child

may "teach" her a lesson and ostensibly protect the child from neglect, but what advantage is that to a child who is left in foster care or in a hospital? The current policy initiatives are examined in light of these perspectives and their potential effectiveness is discussed.

Public Policy Initiatives

The recent increase in public interest has sparked much debate and prompted policy makers to take action. The most comprehensive effort to address the problem of drug abuse was put into law in November of 1988 through the "Anti-Drug Abuse Act of 1988" (P.L. 100-690). While receiving a great deal of publicity as the "war on drugs", the 1988 Act represents an extension of previous federal efforts in dealing with the problem of drug abuse in this country.

The first federal law to control the use of nonmedical drugs was passed as early as 1887 (Hogan & Doyle, 1989). This legislation prohibited the importing of opium into the United States. As is true of many social policies, the control of illegal drugs has been undertaken predominantly by the Federal government. Early efforts were primarily aimed at import control and then expanded to include national restrictions. Provisions for treatment through federal legislation first appeared as part of the Community Mental Health Centers Act of 1963 (P.L. 88-164).

The Comprehensive Crime Control Act of 1984 (P.L. 98-473) included provisions for drug enforcement. It authorized the detention of drug dealers for preventive reasons and introduced more severe penalties for drug trafficking offenses. In terms of policy approaches, it can be characterized as falling under efforts at incarceration and punishment.

In 1986, the Anti-Drug Abuse Act (P.O. 99-570) combined enforcement strategies with treatment efforts. This legislation authorized expanded efforts at enforcement in drug-producing countries, strengthened interdiction efforts and antismuggling provisions, increased criminal sanctions against drug offenders, and authorized increased funding for drug treatment and rehabilitation. The majority of the authorized funding was for law enforcement.

The Anti-Drug Abuse Act of 1988 represents the current strategy endorsed by policy makers. The bill is very comprehensive and includes:

1. establishment of the Office of National Drug Control Policy and creation of the national "drug czar" to serve at the Cabinet level,
2. increases in treatment and prevention programs,
3. international narcotics control efforts,
4. increases criminal penalties,
5. death penalty for major drug traffickers, and
6. stiffer penalties for drug dealers.

The comprehensive nature of the bill acknowledges the complexity of the problem of drug abuse and the wide spread social dimensions. While this breadth of coverage reflects the reality of drug usage, it also presents a potential implementation and coordination nightmare. The legislation includes efforts by eight cabinet departments (Justice, Interior, Health and Human Services, Labor, Education, Treasury, Housing and Urban Development, and Veterans Affairs), the Executive Office of the President, the FBI, the Bureau of Alcohol, Tobacco and Firearms, and numerous other federal offices. Coordination of these efforts is to be run under the newly created Office of National Drug Control Policy. While it is too early to assess the success of this office, the organization of so many high level offices is at best extremely difficult. Already within a year of its implementation, there has been a turnover in the director position bringing to question the stability of the office.

While this bill authorized $5.5 billion to be spent, only $3.7 billion or about 2/3's of the outlined funding was actually committed (Congressional Quarterly Almanac, 1989). Herein lies one of the most frustrating aspects of federal legislative initiatives. While a great deal of publicity surrounds the passage of legislation, little is heard publicly when the actual dollars are committed through federal legislative budgeting.

Of the funds authorized through the 1988 federal legislation, the majority of dollars have been committed to the enforcement and interdiction efforts (U.S. House Select Committee on Children, Youth, & Families, 1990a). In FY 1989, 72% of the funds were earmarked for criminal justice, international control, interdiction, and intelligence. Estimates for FY 1990 and FY 1991 place 71% of these funds set aside for these efforts. Overall, about one fourth of the money authorized by the federal government to deal with the problem of drug abuse is for treatment, education, community services and prevention. Within these treatment funds, about $500 million is for the Al-

cohol and Drug Abuse and Mental Health services Block Grant. Ten percent of the $500 million must be set aside for programs and services designed for women, particularly pregnant women and women with dependent children (Wilson, 1989). Although this is an increase from original set-asides outlined in 1984, it represents a small federal effort in relation to the need.

Thus, while federal policy has expanded in response to the problem of drug abuse, the provisions directed toward the issue of maternal and infant drug dependence represent less than 1% of the total dollars authorized by the 1988 Act. This minuscule amount reflects the federal priority directed towards the problem of women, infants, and drug dependence.

Treatment Efforts

Although there have been earmarked funds to serve women since 1984, the commitment to create and expand programs designed for women has been minimal (Galbraith, 1990). Drug treatment is virtually nonexistent for pregnant women (Fink, 1989). For example, in Michigan, there are 16 licensed programs that provide treatment services for women, but only one serves pregnant drug dependent women in particular (Walker, 1990). Of these programs that do provide services to pregnant women, most have long waiting lists. Of the pregnant women nationwide in need of drug treatment, the National Association of State Alcohol and Drug Abuse Directors estimates that less than 11% receive care (U.S. General Accounting Office, 1990b).

In addition to the inadequate supply of services, a number of barriers act to inhibit the use of those treatment opportunities which do exist. Many drug treatment providers fear the potential liability in serving pregnant women. There is concern about the potential for complications when using medications for treatment, and from the lack of prenatal care. Clinics could be held responsible for consequences suffered by the infant while the mother is being treated. Another basic issue for some women is the need for child care. If there are other children in the family, the lack of child care makes it difficult for a woman to participate in treatment services.

Enforcement Efforts

Pregnant women are reluctant to seek treatment, even when available. There is a growing fear of criminal prosecution if a pregnant woman comes forward and makes public her addiction to illegal drugs. Officials at all levels of government have moved towards im-

plementing more punitive measures against pregnant women who use drugs. These measures include criminalizing drug use during pregnancy and taking custody of newborns who test positive for drugs at birth (Smith, 1990).

The ACLU found that six states have already changed their definition of "neglect" so that infants with controlled substances in their systems at birth are included, and three other states have legislation pending. Hundreds of pregnant women have been charged for child abuse. Over 30 women have been criminally charged for using drugs while pregnant and thus delivering drugs to a minor (U.S. House Select Committee on Children, Youth, and Families, 1990b). These enforcement directions raise a number of concerns regarding civil liberties and open the door to questions of what behaviors by pregnant women are subject to public punishment. Does the drinking of a glass of wine during pregnancy become classified as abusive? Is excessive exercise potentially dangerous to the fetus and thus considered abuse? Not only does this raise questions about what is abuse and neglect, but it helps to ensure that pregnant women who do have a drug problem will not come forward for fear of criminal prosecution and loss of custody of their child.

Education and Prevention

One way to minimize the consequences of drug abuse is to prevent people from becoming addicted in the first place. Prevention is included as part of our current federal policy, but the emphasis is on deterrence through law enforcement. The approach is that if the penalties are high, then a person will not choose to use drugs. This perspective, like the medical model, ignores other variables, such as environmental factors, which contribute to drug use.

Evaluations of programs which stress the terrible consequences of using drugs indicate that they have little or no impact on drug use (Klebe, 1989). Drug information programs alone do not seem to be effective. Rather, programs which promote self-esteem, stress management, and communication skills in addition to providing accurate information seem to have better results.

Implications for Social Work

The growing problem of chemical dependence among women and infants has tremendous ramifications for our social service system. The

social service areas most likely to feel the strain are the child welfare, medical, and educational systems. An increase in the incidence of child abuse and neglect cases can be traced in part to the greater use of drugs. In New York City, cases tripled between 1986 and 1988 and officials identified drugs as a major factor in the increase (Children's Defense Fund, 1990). These changes mean more investigations and cases for an already troubled child welfare system.

In addition to cases of abuse and neglect, a growth in foster care is also attributed in part to the increase in drug abuse. From 1986 to 1989, foster care placements increased by almost 29% nationwide (Besharov, 1990). Over half of the increase is concentrated in two states, California and New York. These two states have been particularly hard hit by crack addiction and insufficient services. The number of drug-exposed infants referred by hospitals to child welfare authorities from 1986 to 1989 rose significantly. In New York, referrals rose by 268%, in Los Angeles by 342%, and in Chicago by 1,735% (U.S. General Accounting Office, 1990b). This phenomenon has given rise to the problem of "boarder babies," infants who remain in a hospital for nonmedical reasons. Foster homes for chemically exposed infants are difficult to find, and often the hospital becomes the only place available for care. Hospitals are becoming providers of child welfare services without additional resources and the appropriate facilities for such care.

Keeping a newborn infant in a hospital for longer periods becomes very costly. Also, drug exposed infants often are in need of intensive care and medical treatment, and this can be very costly. The cost of hospital delivery, perinatal care, and foster care through age 5 is estimated to cost more than $55,000 per child born dependent on cocaine (Office of the Inspector General, 1990). This does not include the potential costs for developmental intervention, education and health care. The first five years of life for a child born drug dependent can cost the social service system well over $225,000. For systems already financially constrained, the increase in the number of babies born to drug addicted mothers will be very costly in the future.

Social Policy Agenda

Current social policy has focused the majority of resources and attention on the enforcement and punishment aspects of controlling drug use. Critics argue that this approach has failed. Although drug enforcement has been the central federal effort, more drugs are avail-

able at cheaper prices, and in fact, U.S. enforcement agencies have stopped only 3-10% of the illegal drugs which enter the country (Moore, 1989). Punitive legal measures will keep pregnant women from using prenatal services and deter them from participating in treatment programs. The fear of liability and insufficient resources continue to limit the availability of services for pregnant women. Continued emphasis on the medical model of treatment which does not address the environmental variables which are related to the use of illicit drugs will perpetuate a focus on symptoms and not cause.

It is crucial that we examine our attitudes and hence approach to dealing with the problem of drug abuse and infants born chemically exposed. The adversarial stance of drug enforcement makes pregnant women and ultimately their babies enemies of the public welfare. Yet, in the end the social welfare system is called to step in and care for the child. The services needed to help a chemical exposed infant are costly and the potential for successful outcomes is poor. A policy approach which is more preventive and less punitive is needed in order to better serve the growing number of babies born to drug using mothers. Such a policy should address the three areas of policy:

Enforcement and Punishment

- Retraction of punitive measures which deter women from seeking help.
- Shifting resources from law enforcement efforts to treatment and preventive efforts.

Treatment

- Treatment services designed especially for women which stress social support and include residential facilities which allow children to stay with their mothers,
- Examination of the environmental strains which are related to drug use and interventions designed to address social conditions which plague areas of heavy drug use,
- Development of community resources which utilize informal adoptions or extended family networks to care for infants in need,
- Expanding the availability of prenatal care for at-risk women.

Education and Prevention

- Drug abuse training for medical and social service professionals,

- Educational materials which are designed for those at greatest risk,
- Teams of specialists trained to do outreach and education to work with community residents to train them to become educators,
- Education programs which are not sensationalist and include efforts to enhance self-esteem and training to promote better stress management, communication skills, and ways to deal with outside social pressures.
- Resources to create employment and educational opportunities for people living in high risk communities.

The incidence of drug dependent babies is unlikely to disappear in the near future. The cost of caring for these children is high and is taxing our already strained social welfare system. Measures which reach women early and provide care and support have a better likelihood of creating a positive intervention. Overall, it is also extremely important to remember the social factors which contribute to the abuse of drugs. Lack of economic opportunity, social isolation, poor education, and poverty all contribute to making the use and selling of drugs attractive. These social concerns must also be addressed if headway is to be gained in decreasing the incidence of drug abuse in this country.

References

Atkins, W. T. (1990). [Testimony before the U.S. House Select Committee on Children, Youth, & Families]. *Getting straight: Overcoming treatment barriers for addicted women and their children.* Washington, DC: U.S. Government Printing Office.

Besharov, D. J. (1990). [Testimony before the U.S. House Select Committee on Children, Youth, & Families]. *Beyond the stereotypes: Women, addiction, and perinatal substance abuse.* Washington, DC: U.S. Government Printing Office.

Chasnoff, I. J. (1989). Drug use and women: Establishing a standard of care. *Annals of the New York Academy of Science, 562,* 208-210.

Children's Defense Fund. (1990). *SOS America.* Washington, DC: Author.

Congressional Quarterly Almanac. (1989). Volume XLV, 252-258. Washington, DC: Congressional Quarterly, Inc.

Fink, J. R. (1989). Effects of crack and cocaine upon infants: A brief review of the literature. *Children's Legal Rights Journal, 10*(4), 4-12.

Galbraith, S. (1990). [Testimony before the U.S. House Select Committee on Children, Youth, & Families]. *Law and policy affecting addicted women and their children.* Washington, DC: U.S. Government Printing Office.

Hogan, H. & Doyle, C. (1989). The federal response: A growing role. *CRS Review, 10*(10), 11-13.

Johnson, H. L. & Rosen, T. S. (1990). Mother-infant interaction in a multirisk population. *American Journal of Orthopsychiatry, 60*(2), 281-288.

Klebe, E. R. (1989). The federal response: A growing role. *CRS Review, 10*(10), 14-16.
Moore, W. J. (1989). Dissenters in the drug war. *National Journal, 21*, 2692-2695.
National Institute on Drug Abuse. (1989). *Drug use, drinking, and smoking: National survey results from high school, college, and young adult populations* (DHHS Publication No. ADM89-1638). Washington, DC: U.S. Government Printing Office.
O'Brien, R. & Cohen, S. (1984). *The encyclopedia of drug abuse.* New York: Facts on File, Inc.
Office of the Inspector General. (1990). *Crack babies* (Report No. OEI-03-89-01540). Washington, DC: Author.
Peele, S. (1989). *Diseasing of America.* Lexington, MA: Lexington Books.
Segal, B. (1988). *Drugs and behavior.* New York: Gardner Press, Inc.
Smith, B. V. (1990). [Testimony before the U.S. House Select Committee on Children, Youth, & Families]. *Law and policy affecting addicted women and their children.* Washington, DC: U.S. Government Printing Office.
U.S. General Accounting Office. (1990a). *Drug control* (Report No. GAO/NSIAD-90-133). Washington, DC: Author.
U.S. General Accounting Office. (1990b). *Drug-exposed infants* (Report No. GAO/HRD-90-138). Washington, DC: Author.
U.S. House Select Committee on Children, Youth, & Families. (1990a). *Beyond the stereotypes: Women, addiction, and perinatal substance abuse.* Washington, DC: U.S. Government Printing Office.
U.S. Select Committee on Children, Youth, & Families. (1990b). *Getting straight: Overcoming treatment barriers for addicted women and their children.* Washington, DC: U.S. Government Printing Office.
Walker, J. (1990). [Testimony before the U.S. House Select Committee on Children, Youth, & Families]. *Getting straight: Overcoming treatment barriers for addicted women and their children.* Washington, DC: U.S. Government Printing Office.
Wilson, J. J. (1989). *The Anti-Drug Abuse Act of 1988 treatment and prevention programs.* Washington, DC: National Conference of State Legislatures.